

Cross-talk between disulfidptosis and immune check point genes defines the tumor microenvironment for the prediction of prognosis and immunotherapies in glioblastoma.

PMID: 38365809 | Indexed in PubMed

Disulfidptosis is a condition where dysregulated NADPH levels and abnormal accumulation of cystine and other disulfides occur in cells with high SLC7A11 expression under glucose deficiency. This disrupts normal formation of disulfide bonds among cytoskeletal proteins, leading to histone skeleton collapse and triggering cellular apoptosis. However, the correlation between disulfidptosis and immune responses in relation to glioblastoma survival rates and immunotherapy sensitivity remains understudied. Therefore, we utilized The Cancer Genome Atlas and The Chinese Glioma Genome Atlas to identify disulfidptosis-related immune checkpoint genes and established an overall survival (OS) prediction model comprising six genes: CD276, TNFRSF 14, TNFSF14, TNFSF4, CD40, and TNFRSF18, which could also be used for predicting immunotherapy sensitivity. We identified a cohort of glioblastoma patients classified as high-risk, which exhibited an upregulation of angiogenesis, extracellular matrix remodeling, and epithelial-mesenchymal transition as well as an immunosuppressive tumor microenvironment (TME) enriched with tumor associated macrophages, tumor associated neutrophils, CD8 + T-cell exhaustion. Immunohistochemical staining of CD276 in 144 cases further validated its negative correlation with OS in glioma. Disulfidptosis has the potential to induce chronic inflammation and an immunosuppressive TME in glioblastoma.

Rare case report of moxifloxacin-induced persistent hiccups.

PMID: 37369596 | Indexed in PubMed

Moxifloxacin is a broad-spectrum antimicrobial agent that is commonly used in clinical practice. Here we report an unusual case of a patient with persistent hiccups caused by moxifloxacin. A man aged in his 40s was treated with moxifloxacin for tuberculous pleurisy. Hiccups occurred 2 hours after intravenous injection of moxifloxacin and lasted into evening. On the second day after injection, hiccups occurred again and made it difficult for him to fall asleep. The clinician ruled out gastrointestinal disease, nervous system disease, electrolyte disturbance and other factors. On assessing causality of the adverse drug reaction, the Naranjo scale for moxifloxacin was six, indicating a probable relationship of hiccups with moxifloxacin. Hiccups stopped 2 min after intramuscular injection of metoclopramide. To our knowledge, this is the first case report about moxifloxacin-induced persistent hiccups. Clinicians should be aware of the rare adverse reaction.

Antibiotic-Induced Hiccups: A Case Report and Brief Literature Review.

PMID: 38450348 | Indexed in PubMed

Hiccups, also called hiccoughs, are sudden, involuntary and rapid expulsion of air from the lungs with synchronous closure of the glottis causing blockade of the air flow. Hiccups may be induced by a multitude of etiologies such as central nervous disorders, gastrointestinal disorders, cardiovascular disorders, psychogenic factors, and metabolic disorders. Hiccups induced by medications are rare. The diagnosis of drug-induced hiccup is difficult. The exact mechanism responsible for this adverse drug reaction is still unknown. Herein, we report the first case of cefotaxime-induced hiccups and briefly

review the literature on antibiotic-induced hiccups.

Possible azithromycin-associated hiccups.

PMID: 15985056 | Indexed in PubMed

To report a case of persistent hiccups associated by azithromycin therapy. A 76-year-old man presented with persistent hiccups after beginning azithromycin for the treatment of pharyngitis. Hiccups were persistent and exhausting. Discontinuation of azithromycin and therapy with baclofen finally resolved hiccups. No organic cause of hiccups was identified despite extensive investigation. Pharmacotherapeutic agents have been uncommonly associated with hiccups. Corticosteroids (dexamethasone and methylprednisolone), benzodiazepines (midazolam) and general anaesthesia have been the specific agents mentioned most frequently in the literature as being associated with the development of hiccups. Few cases of drug-induced hiccups have been reported related to macrolide antimicrobials. Using the Naranjo adverse effect reaction probability scale this event could be classified as possible (score 5 points), mostly because of the close temporal sequence, previous reports on this reaction with other macrolides and the absence of any alternative explanation for hiccups. Our hypothesis is that a vagal mechanism mediated by azithromycin could be the pathogenesis of hiccups in our patient. Diagnosis of drug-induced hiccups is difficult and often achieved only by a process of elimination. However, macrolide antimicrobials have been reported to be associated with hiccups and vagal mechanism could explain the development of this side-effect.

Short report: severe hiccups secondary to doxycycline-induced esophagitis during treatment of malaria.

PMID: 8619448 | Indexed in PubMed

A 51-year-old man who was treated with quinine and doxycycline for *Plasmodium falciparum* malaria acquired in West Africa developed hiccups soon after his first dose of antimalarial therapy. Endoscopic examination performed when his hiccups became intractable showed an esophageal erosion and ulcer most likely due to doxycycline. The patient's symptoms resolved on treatment with omeprazole and sucralfate.

Dopamine Agonist-Associated Hiccup in Parkinson's Disease: A Case Report.

PMID: 34526851 | Indexed in PubMed

Hiccup is described as the sudden involuntary contraction of the diaphragm, and the intercostal muscles followed by the immediate closure of the glottis. Corticosteroids, benzodiazepines, and antibiotics may cause drug-induced hiccups. Dopamine agonist-induced hiccups were reported in patients diagnosed with Parkinson's Disease (PD) in small number of cases. Here we report a patient diagnosed with PD who had severe hiccups with the use of two dopamine agonists in treatment, however hiccup was not reported with the use of Levodopa. This information may help to manage the treatment of PD, and avoid the unnecessary diagnostic procedures.

Surfactant therapy using vibrating-mesh nebulizers in adults with COVID-19-induced ARDS: A case series.

PMID: 38444695 | Indexed in PubMed

Coronavirus adult respiratory distress syndrome, characterized by decreased surfactant due to lysis of type II pneumocytes and hyaline membrane formation, contributes to severe hypoxemia. The administration of surfactant via high-flow nasal cannula (HFNC) may positively affect lung structure and function in this context. In this study, we report on five clinical cases, encompassing patients aged 40-60 years of both sexes, who tested positive for coronavirus disease 2019 via real-time polymerase chain reaction and exhibited significant pulmonary compromise with elevated inflammatory biomarkers. These patients were treated with aerosol therapy using surfactant delivered through vibrating-mesh nebulizers alongside HFNC. Of these patients, four demonstrated positive responses to the treatment, suggesting that aerosol therapy with surfactant through vibrating-mesh nebulizers could be a viable rescue therapy in adults receiving HFNC oxygen therapy for hypoxemic respiratory failure caused by severe acute respiratory syndrome coronavirus 2 (SARS-CoV-2). Unfortunately, one patient had a negative outcome and succumbed. The findings from these cases indicate that the use of aerosol therapy with vibrating-mesh nebulizers as rescue therapy might offer an alternative approach for managing adults with hypoxemic respiratory failure due to SARS-CoV-2, as evidenced by the positive outcomes in four out of the five cases presented.

The Use of Exogenous Lung Surfactant (Poractant Alfa) in Acute Respiratory Failure by Drowning.

PMID: 32566323 | Indexed in PubMed

Drowning is an acute respiratory failure as a result from immersion or submersion of the airways in a liquid medium (predominantly water). Inhalation of water causes severe lung damage due to the destruction of pulmonary surfactant, resulting in decreased lung elasticity, alveolar collapse, alteration of ventilation-perfusion ratio, intrapulmonary blood shunting, hypoxia, acute lung injury, and Acute Respiratory Distress Syndrome (ARDS). Poractant alfa (Curosurf®), a natural surfactant effective in the treatment of newborn respiratory distress, has been used in various forms of ARDS, but in drowning syndromes, experience is still poor. We describe a series of nine clinical cases of drowning, six adults and three children, treated in our Intensive Care Unit (ICU) with endobronchial administration of poractant alfa. After 24 and 48 hours of administration in all cases, there was an improvement in arterial blood gas analysis (ABG) parameters and imaging. All patients were discharged without clinical consequences.

Use of natural surfactant in an infant with aspiration syndrome.

PMID: 17978790 | Indexed in PubMed

The remarkable effectiveness of surfactant in neonatology suggested its use also in adults as well as in children on the basis of the pulmonary pathophysiology. We describe a case of an 18 month-old child affected by acute respiratory distress syndrome (ARDS) due to asphyxy by gastric juice inhalation,

successfully treated with porcine surfactant (Curosurf) associated with ventilatory therapy.

Advances in neonatal care and surgery.

PMID: 17013236 | Indexed in PubMed

Neonatology has seen many advances over the past decade. Exogenous surfactant therapy is now a mainstay treatment for respiratory distress syndrome. Partial liquid ventilation, high-frequency ventilation, and inhaled nitric oxide are all relatively new modalities, which have enabled neonatologists to treat with varying degrees of success ever younger and smaller patients. The purpose of this review is to examine studies regarding the long-term outcome of high-risk neonates, the various treatment modalities, and current neonatal surgical techniques, all of which will influence our care of the neonate.
